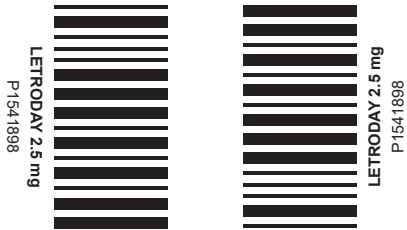


Pharmacode position may change as per Supplier’s m/c requirement & additional small pharma code may appear on the front / back panel



SUMMARY OF PRODUCT CHARACTERISTICS

LETRODAY 2.5
Letrozole Tablets 2.5 mg

Rx Only

NAME OF THE PRODUCT: Letrozole Tablets 2.5 mg
(TRADE) NAME OF PRODUCT: LETRODAY 2.5

STRENGTH: 2.5 mg

PHARMACEUTICAL DOSAGE FORM: Film Coated Tablet

QUALITATIVE AND QUANTITATIVE COMPOSITIONS:

Each film-coated tablet contains:
Letrozole Ph.Eur 2.5 mg

PHARMACEUTICAL FORM

Letrozole Tablets 2.5 mg
Dark yellow, film coated, round, slightly biconvex, with beveled edges tablet debossed with 'L2.5' on one side and plain on the other side.

CLINICAL PARTICULARS

Therapeutic indications

- Adjuvant treatment of postmenopausal women with hormone receptor positive early breast cancer.
- Adjuvant treatment of postmenopausal women with early breast cancer (positive or unknown oestrogen or progesterone receptor status) who have received 5 years of adjuvant tamoxifen therapy (extended adjuvant therapy).
- First-line treatment in postmenopausal women with hormone-dependent advanced breast cancer.
- Treatment of advanced breast cancer in women with natural or artificially induced postmenopausal status, who have previously been treated with antioestrogens.
- Pre-operative therapy in postmenopausal women with localised hormone receptor positive breast cancer, to allow subsequent breast-conserving surgery in women not originally considered candidates for this type of surgery. Subsequent treatment after surgery should be in accordance with standard of care.

Posology and method of administration

Posology

General target population

Adults

The recommended dose of LETRODAY is 2.5 mg once daily. In the adjuvant and extended adjuvant setting, treatment with LETRODAY should continue for 5 years or until disease relapse/recurrence occurs, whichever comes first. In the large pivotal study of letrozole versus tamoxifen in the adjuvant setting, no benefit in efficacy or safety was obtained by sequential administration of these treatments compared with letrozole administered continuously for 5 years. In patients with metastatic disease, treatment with LETRODAY should continue until tumor progression is evident. In the pre-operative setting, treatment with LETRODAY should be continued for 4 to 8 months in order to establish optimal tumor reduction. If the response is not adequate, treatment with LETRODAY should be discontinued, surgery scheduled and/or further treatment options discussed with the patient.

Special populations

Hepatic impairment

No dose adjustment of LETRODAY is required for patients with mild to moderate hepatic insufficiency (Child-Pugh score A or B). Insufficient data are available for patients with severe hepatic impairment, but patients with severe hepatic impairment (Child-Pugh score C) should be kept under close supervision (see sections WARNINGS AND PRECAUTIONS and CLINICAL PHARMACOLOGY - Pharmacokinetics).

Renal impairment

No dosage adjustment of LETRODAY is required for patients with renal insufficiency with (creatinine clearance (CLcr ≥10 mL/min.). Insufficient data are available in cases of renal insufficiency with CLcr <10mL/min (see sections WARNINGS ANDPRECAUTIONS and CLINICAL PHARMACOLOGY -Pharmacokinetics).

Pediatrics

LETRODAY is not recommended for use in children and adolescents. The safety and efficacy of LETRODAY in children and adolescents aged up to 17 years have not been established. Limited data are available and no recommendation on a posology can be made.

Geriatrics

No dose adjustment is required for elderly patients.

Missed dose

The missed dose should be taken as soon as the patient remembers. However, if it is almost time for the next dose, the missed dose should be skipped, and the patient should go back to her regular dosage schedule. Doses should not be doubled because with daily doses over the 2.5 mg recommended dose, over-proportionality in systemic exposure was observed (see section CLINICAL PHARMACOLOGY).

Method of administration

Letrozole should be taken orally and can be taken with or without food.

Contraindications

- Hypersensitivity to the active substance or to any of the Excipients.
- Premenopausal endocrine status
- Pregnancy
- Breast-feeding

Special warnings and precautions for use

Menopausal status

In patients whose menopausal status is unclear, luteinising hormone (LH), follicle-stimulating hormone (FSH) and/or oestradiol levels should be measured before initiating treatment with LETRODAY. Only women of postmenopausal endocrine status should receive LETRODAY.

Renal impairment

LETRODAY has not been investigated in a sufficient number of patients with a creatinine clearance lower than 10 mL/min. The potential risk/benefit to such patients should be carefully considered before administration of LETRODAY.

Hepatic impairment

In patients with severe hepatic impairment (Child-Pugh C), systemic exposure and terminal half-life were approximately doubled. Such patients should therefore be kept under close supervision.

Bone effects

LETRODAY is a potent oestrogen-lowering agent. Women with a history of osteoporosis and/or fractures, or who are at increased risk of osteoporosis, should have their bone mineral density formally assessed prior to the commencement of adjuvant and extended adjuvant treatment and monitored during and following treatment with letrozole. Treatment or prophylaxis for osteoporosis should be initiated as appropriate and carefully monitored. In the adjuvant setting a sequential treatment schedule (letrozole 2 years followed by tamoxifen 3 years) could also be considered depending on the patient's safety profile.

Tendonitis and tendon rupture

Tendonitis and tendon ruptures (rare) may occur. Close monitoring of the patients and appropriate measures (e.g. immobilisation) must be initiated for the affected tendon.

Other warnings

Co-administration of LETRODAY with tamoxifen, other anti-oestrogens or oestrogen-containing therapies should be avoided as these substances may diminish the pharmacological action of letrozole.

As the tablets contain lactose, LETRODAY is not recommended for patients with rare hereditary problems of galactose intolerance, of severe lactase deficiency or of glucose-galactose malabsorption

Interaction with other medicinal products and other forms of interaction

Metabolism of letrozole is partly mediated via CYP2A6 and CYP3A4. Cimetidine, a weak, unspecific inhibitor of CYP450 enzymes did not affect the plasma concentrations of letrozole. The effect of potent CYP450 inhibitors is unknown.

There is no clinical experience to date on the use of Letrozole in combination with oestrogens or other anticancer agents, other than tamoxifen. Tamoxifen, other anti-oestrogens or oestrogen-containing therapies may diminish the pharmacological action of letrozole. In addition, co-administration of tamoxifen with letrozole has been shown to substantially decrease plasma concentrations of letrozole. Co-administration of letrozole with tamoxifen, other anti-estrogens

or oestrogens should be avoided.

In vitro, letrozole inhibits the cytochrome P450 isoenzymes 2A6 and, moderately, 2C19, but the clinical relevance is unknown. Caution is therefore indicated when giving letrozole concomitantly with medicinal products whose elimination is mainly dependent on these isoenzymes and whose therapeutic index is narrow (e.g. phenytoin, clopidrogel).

Fertility, pregnancy and lactation

Women of perimenopausal status or child-bearing potential

Letrozole should only be used in women with a clearly established postmenopausal status. Women may regain ovarian function during treatment with Letrozole despite a clear postmenopausal status atstart of therapy, the physician needs to discuss adequate contraception when necessary.

Pregnancy

Letrozole may cause congenital malformations when administered during pregnancy. Studies in animals have shown reproductive toxicity.

Letrozole is contraindicated during pregnancy.

Breast-feeding

It is unknown whether letrozole and its metabolites are excreted in human milk. A risk to the newborns/ infants cannot be excluded.

Letrozole is contraindicated during breast-feeding.

Fertility

The pharmacological action of letrozole is to reduce oestrogen production by aromatase inhibition. In premenopausal women, the inhibition of oestrogen synthesis leads to feedback increases in gonadotropin (LH, FSH) levels. Increased FSH levels in turn stimulate follicular growth, and can induce ovulation.

Effects on ability to drive and use machines

LETRODAY has minor influence on the ability to drive and use machines. Since fatigue and dizziness have been observed with the use of LETRODAY and somnolence has been reported uncommonly, caution is advised when driving or using machines.

Summary of the safety profile

Side Effects

The most frequently reported adverse reactions were hot flushes, hypercholesterolaemia, arthralgia, fatigue, increased sweating and nausea.

Important additional adverse reactions that may occur with LETRODAY are: skeletal events such as osteoporosis and/or bone fractures and cardiovascular events (including cerebrovascular and thromboembolic events). The frequency category for these adverse reactions is described in Table 1.

Table 1

Infections and infestations	
Uncommon	Urinary tract infection
Neoplasms benign, malignant and unspecified (including cysts and polyps)	
Uncommon	Tumour pain
Blood and lymphatic system disorders	
Uncommon	Leukopenia
Immune system disorders	
Not known	Anaphylactic reaction
Metabolism and nutrition disorders	
Very common	Hypercholesterolemia
Common	Decreased appetite, increased appetite
Psychiatric disorders	
Common	Depression
Uncommon	Anxiety (including nervousness), irritability

A/s : 210 X 300 mm

 Packaging Development		Product Name LETRODAY 2.5 mg (Letrozole Tablets 2.5 mg)	Component Leaflet	Item Code P1541898	Date & Time 04.03.2026 10:00 am
		Customer / Country Malaysia_Eugia	Version No. 00	Reason Of Issue Submission	Reviewed / Approved by Sign / Date
Team Leader Narender	Dimensions A/s : 210 X 300 mm	No. of Colours : 01			
Initiator Sai Kalyan	Pharma Code 41898	 41898			
Artist:  Ashok					
Additional Information: Supersede Code: P1521342					

Nervous system disorders	
Common	Headache, dizziness
Uncommon	Somnolence, insomnia, memory impairment, dysaesthesia (including paraesthesia, hypoaesthesia), dysgeusia, cerebrovascular accident, carpal tunnel syndrome
Eye disorders	
Uncommon	Cataract, eye irritation, blurred vision
Cardiac disorders	
Common	Palpitations
Uncommon	Tachycardia, ischaemic cardiac events (including new or worsening angina, angina requiring surgery, myocardial infarction and myocardial ischaemia)
Vascular disorders	
Very common	Hot flushes
Common	Hypertension
Uncommon	Thrombophlebitis (including superficial and deep vein thrombophlebitis)
Rare	Pulmonary embolism, arterial thrombosis, cerebral infarction
Respiratory, thoracic and mediastinal disorders	
Uncommon	Dyspnoea, cough
Gastrointestinal disorders	
Common	Nausea, dyspepsia, constipation, abdominal pain, diarrhoea, vomiting
Uncommon	Dry mouth, stomatitis
Hepatobiliary disorders	
Uncommon	Increased hepatic enzymes, hyperbilirubinemia, jaundice
Not known	Hepatitis
Skin and subcutaneous tissue disorders	
Very common	Hyperhidrosis
Common	Alopecia, rash (including erythematous, maculopapular, psoriaform, and vesicular rash), dry skin
Uncommon	Pruritus, urticaria
Not known	Angioedema, toxic epidermal necrolysis, erythema multiforme
Musculoskeletal and connective tissue disorders	
Very common	Arthralgia
Common	Myalgia, bone pain, osteoporosis, bone fractures, arthritis
Uncommon	Tendonitis
Rare	Tendon rupture
Not known	Trigger finger
Renal and urinary disorders	
Uncommon	Poliakiuria
Reproductive system and breast disorders	
Common	Vaginal haemorrhage
Uncommon	Vaginal discharge, vulvovaginal dryness, breast pain
General disorders and administration site conditions	
Very common	Fatigue (including asthenia, malaise)
Common	Peripheral oedema, chest pain
Uncommon	General oedema, mucosal dryness, thirst, pyrexia
Investigations	
Common	Weight increased
Uncommon	Weight decreased

Overdose

No specific treatment for overdose is known; treatment should be symptomatic and supportive.

PHARMACOLOGICAL PROPERTIES

Pharmacotherapeutic group: Endocrine therapy. Hormone antagonist and related agents: aromatase inhibitor, ATC code:L02BG04.

Pharmacodynamic effects

The elimination of oestrogen-mediated growth stimulation is a prerequisite for tumour response in cases where the growth of tumour tissue depends on the presence of oestrogens and endocrine therapy is used. In postmenopausal women, oestrogens are mainly derived from the action of the aromatase enzyme, which converts adrenal androgens -primarily androstenedione and testosterone - to oestrone and oestradiol. The suppression of oestrogen biosynthesis in peripheral tissues and the cancer tissue itself can therefore be achieved by specifically inhibiting the aromatase enzyme.

Letrozole is a non-steroidal aromatase inhibitor. It inhibits the aromatase enzyme by competitively binding to the haem of the aromatase cytochrome P450, resulting in a reduction of oestrogen biosynthesis in all tissues where present.

Letrozole is highly specific in inhibiting aromatase activity. It does not impair adrenal steroidogenesis. Glucocorticoid and mineralocorticoid supplementation is not necessary.

Plasma levels of LH and FSH are not affected by letrozole, nor is thyroid function as evaluated by TSH, T4, and T3 uptake test.

Male breast cancer

Use of Letrozole in men with breast cancer has not been studied.

Pharmacokinetic properties

Absorption

Letrozole is rapidly and completely absorbed from the gastrointestinal tract (mean absolute bioavailability: 99.9%). Food slightly decreases the rate of absorption (median tmax 1 hour fasted versus 2 hours fed; and mean Cmax 129 ± 20.3nmol/litre fasted versus 98.7 ± 18.6 nmol/litre fed) but the extent of absorption (AUC) is not changed. The minor effect onthe absorption rate is not considered to be of clinical relevance, and therefore letrozole may be taken without regard to meal times.

Distribution

Plasma protein binding of letrozole is approximately 60%, mainly to albumin (55%). The concentration of letrozole in erythrocytes is about 80% of that in plasma. Systemic exposure to metabolites is therefore low. Letrozole is rapidly and extensively distributed to tissues. Its apparent volume of distribution at steady state is about 1.87 ± 0.47 l/kg.

Biotransformation

Metabolic clearance to a pharmacologically inactive carbinol metabolite is the major elimination pathway of letrozole (CLm = 2.1 l/h) but is relatively slow when compared to hepatic blood flow (about 90 l/h). The cytochrome P450 isoenzymes 3A4 and 2A6 were found to be capable of converting letrozole to this metabolite. Formation of minor unidentified metabolites and direct renal and faecal excretion play only a minor role in the overall elimination of letrozole.

Elimination

The apparent terminal elimination half-life in plasma is about 2 to 4 days. After daily administration of 2.5 mg steady state levels are reached within 2 to 6 weeks. Plasma concentrations at steady state are approximately 7 times higher than concentrations measured after a single dose of 2.5 mg, while they are 1.5 to 2 times higher than the steady-state values predicted from the concentrations measured after a single dose, indicating a slight non-linearity in the pharmacokinetics of letrozole upon daily administration of 2.5 mg. Since steady-state levels are maintained over time, it can be concluded that no continuous accumulation of letrozole occurs.

Linearity/ non-linearity

The pharmacokinetics of letrozole were dose proportional after single oral doses up to 10 mg (dose range: 0.01 to 30 mg) and after daily doses up to 1.0 mg (dose range: 0.1 to 5mg). After a 30 mg single oral dose there was a slightly dose over-proportional increase in AUC value. The dose over-proportionality is likely to be the result of a saturation of metabolic elimination processes. Steady levels were reached after 1 to 2 months at all dosage regimens tested (0.1-5.0 mg daily).

Special populations

Elderly

Age had no effect on the pharmacokinetics of letrozole.

Renal Impairment

No effect on the pharmacokinetics of letrozole was observed after a single dose of 2.5 mg. No evidence of an adverse effect of letrozole on CLcr or an impairment of renal function.

Therefore, no dose adjustment is required for patients with renal impairment (CLcr ≥10 mL/min). Little information is available in patients with severe impairment of renal function (CLcr <10 mL/min).

Hepatic impairment

Letrozole should be administered with caution topatients with severe hepatic impairment and after consideration of the risk/benefit in the individual patient.

PHARMACEUTICAL PARTICULARS

List of excipients

Lactose monohydrate, Cellulose microcrystalline, Sodium starch glycolate (Type A), Maize starch, Silica colloidal anhydrous, Magnesium stearate, Opadry yellow and Purified Water.

Incompatibilities

Not applicable

Shelf life

Please refer outer package for expiry date.

Special precautions for storage

Do not store above 30°C.Store in original pack and protect from light.

Nature and contents of container

Blister of (3x10's) tablets in carton box

Manufactured by:

Eugia Pharma Specialities Limited, Unit-1, Survey No. 550, 551 & 552, Kolthur Village, Shamirpet Mandal, Medchal-Malkajgiri District, Shamirpet, Telangana, 500101, India.

Product Registration Holder

Synerrv Sdn Bhd, SO-29-2, Menara 1, KL ECO City, Jalan Bangsar, KG Haji Abdullah Hukum, 59200 Kuala Lumpur Wilayah Persekutuan Kuala Lumpur, Malaysia

DATE OF PREPARATION OF THIS LEAFLET

March 2026